CONSULTATION REASON: Evaluation of progressive resting tremor in the right hand, slowing of daily motor activities, and gait instability over 18 months.

HISTORY OF PRESENT ILLNESS: A 67-year-old retired schoolmaster noticed an asymmetric, intermittent 4-to-5 Hz resting tremor involving his right thumb and index finger ('pill-rolling' tremor) that disappears during voluntary motor tasks. He reports significant bradykinesia: dressing and buttoning shirts takes twice as long, and his handwriting has become progressively small and illegible (micrographia). His family notes a reduction in his facial expressiveness, softened vocal volume (hypophonia), and a forward-stooped posture when walking. Review of non-motor symptoms is positive for anosmia (loss of smell for 3 years), chronic constipation, and vivid dream enactment behavior during sleep (consistent with REM sleep behavior disorder).

NEUROLOGICAL ASSESSMENT:

General Appearance: Bilateral facial hypomimia with reduced spontaneous blink rate (masked facies).

Speech: Monotonous, hypophonic dysarthria.

Motor Tone: Cogwheel rigidity elicited in the right wrist and elbow, which is markedly reinforced by voluntary activation of the contralateral limb (Froment sign positive). Mild lead-pipe rigidity in the right lower extremity.

Tremor: Characteristic 4-5 Hz resting tremor observed in the right upper extremity, suppressed upon finger-to-nose targeting, with re-emergence after posture holding.

Bradykinesia: Significant progressive decrement in speed and amplitude during repetitive finger tapping, hand opening-closing, and rapid alternating pronation-supination on the right side compared to the left.

Gait and Stance: Stooped posture with reduced arm swing on the right. Shuffling steps with narrow base and mild festination when turning. Pull test (postural instability): Takes three retropulsive compensatory steps upon sudden shoulder displacement.

IMPRESSION AND CLINICAL PLAN:

1. Idiopathic Parkinson's Disease (Hoehn and Yahr Stage II: Bilateral motor involvement without balance impairment).

2. Initiate dopaminergic replacement therapy with oral Carbidopa/Levodopa (Sinemet 25/100 mg) one-half tablet three times daily for one week, advancing to one full tablet TID.

3. Prescribe physical therapy with LSVT-BIG program focused on high-amplitude functional movement training and fall prevention.

4. Encourage daily aerobic exercise and hydration with dietary fiber for autonomic bowel management.